



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

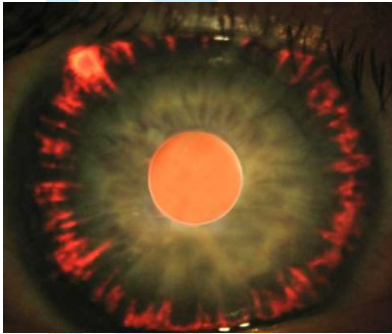
Q1. A lady presents with history of recurrent episodes of amaurosis fugax and headache. On examination, pupillary reactions and anterior segment is normal. Fundus examination shows bilateral optic disc edema. Neuroimaging is done and is normal. Which of the following is the most likely diagnosis in this case?

- a) Retrobulbar neuritis
- b) Idiopathic intracranial hypertension
- c) Intracranial tumor
- d) Orbital lymphoma

Q2. Which of the following drugs decrease intraocular pressure by increasing the uveoscleral outflow?

- a) Netarsudil
- b) Acetazolamide
- c) Travoprost
- d) Timolol

Q3. A patient is found to have the findings shown in the given image on slit lamp examination. Which of the following is not true of this condition?



- a) Krukenberg spindle is seen on cornea.
- b) Patient can present with open angle glaucoma.
- c) Patient can develop angle closure glaucoma.
- d) Iris atrophy is seen.

Q4. A young girl presents with a painful swelling in upper lid located away from the lid margin. Which of the following glands are likely to be involved in this case?

- a) Meibomian glands
- b) Glands of Zeis
- c) Glands of Moll
- d) Lacrimal gland

Q5. A baby girl is brought by her mother to eye OPD with drooping of upper lid in her left eye since birth. The lid drooping improves when baby is being fed. What is the most likely cause of her condition?

- a) Congenital third nerve palsy
- b) Blepharophimosis epicanthus inversus syndrome
- c) Congenital ankyloblepharon
- d) Marcus Gunn synkinetic ptosis



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Q6. Which of the following is the drug of choice for cycloplegic refraction in a 2 years old child with squint?

- a) Atropine 1% eye drops
- b) Atropine 1% eye ointment
- c) Homatropine
- d) Cyclopentolate

Q7. The test shown in the given image is done for:



- a) Test for Stereopsis
- b) Color vision test
- c) Contrast sensitivity test
- d) Test for Diplopia

Q8. A young man reports difficulty in night vision and loss of peripheral field. On examination, following fundus findings are seen. What is the most appropriate diagnostic test in this condition?



- a) Fundus fluorescein angiography
- b) Electroretinogram
- c) Visual evoked response
- d) Electrooculogram

Q9. Which of the following medium can be used for preserving corneoscleral graft for 48 hours?

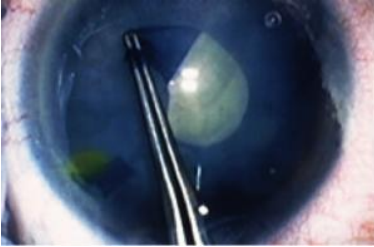
- a) Sabouraud dextrose agar medium
- b) Organ culture culture
- c) McCarey Kaufman medium
- d) Nutrient agar medium

Q10. Identify the structure being stained and dye used in the given image.



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026



- a) Cornea with sodium fluorescein
- b) Lens capsule with lissamine green
- c) Lens capsule with trypan blue
- d) Conjunctiva with trypan blue

Q11. A 6 years old girl is diagnosed to have an uncorrected refractive error of +4.00Dsph in right eye and +0.5Dsph in left eye. Which of the following will be seen in this case?

- a) Anisometropic amblyopia
- b) Sensory deprivation amblyopia
- c) Anisokenia
- d) Diplopia

Q12. A soft contact lens user presents with severely painful eye and decreased vision. On slit lamp examination, following findings are seen. What could be the likely diagnosis?



- a) Disciform keratitis
- b) Acanthamoeba keratitis
- c) Pseudomonas keratitis
- d) Peripheral ulcerative keratitis

Q13. Which of the following will be the treatment of choice for a patient who had a trauma to the eye with a leaf 2 days back and presents with the given slit lamp findings?



- a) Acyclovir ointment
- b) Natamycin eye drops



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

- c) Moxifloxacin eye drops
- d) Steroid eye drops

Q14. A farmer presents with decreased vision and a progressively increasing fleshy growth in the eye as shown below. What is the most appropriate management in this case?



- a) Anti-allergic drops
- b) Excision with conjunctival autograft
- c) Observation and use of UV protection glasses
- d) Simple Excision

Q15. Which of the following provides the best prophylaxis against postoperative endophthalmitis after cataract surgery?

- a) Preoperative antibiotics
- b) Preoperative povidone-iodine
- c) Postoperative antibiotics
- d) Intraoperative antibiotics

Q16. A 17-year-old boy presents to casualty at 2 AM with sudden-onset severe right scrotal pain that woke him from sleep. He has nausea and two episodes of vomiting. There is no fever, dysuria, urethral discharge, trauma, or previous urinary infection.

On examination:

- Pulse: 112/min
- Temperature: 37°C
- Right testis is high-riding and horizontally placed
- Cremasteric reflex is absent on the right side
- Prehn sign is negative
- Doppler ultrasound is not immediately available

What is the most appropriate next step?

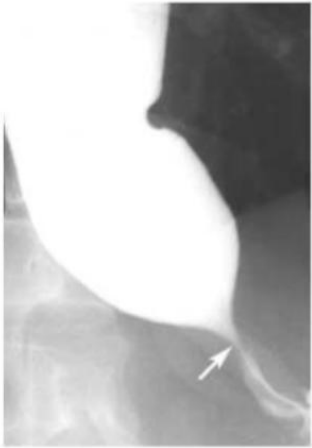
- a) Start IV antibiotics and NSAIDs; review after 24 hours
- b) Immediate scrotal exploration with bilateral orchidopexy
- c) Wait for Doppler confirmation before surgery
- d) Manual detorsion followed by discharge if pain improves

Q17. A 38-year-old woman presents with long-standing dysphagia to both solids and liquids. She reports nocturnal regurgitation of undigested food and recurrent episodes of aspiration pneumonia. Barium swallow demonstrates a "bird-beak" appearance.



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026



The primary pathophysiological defect is:

- a) Hypertrophy of cricopharyngeus muscle
- b) Loss of ganglion cells in Auerbach's plexus
- c) Fibrosis of lower esophageal sphincter
- d) Autoimmune destruction of gastric parietal cells

Q18. A 64-year-old man presents with painless jaundice and weight loss. CT shows a pancreatic body mass involving the celiac axis. There is no liver metastasis.

Which statement is most appropriate?

- a) Whipple procedure is the standard surgery
- b) Distal pancreatectomy is always curative
- c) Celiac axis involvement usually indicates unresectability
- d) Simple cystogastrostomy is indicated

Q19. A 56-year-old man presents with rectal bleeding. Colonoscopy shows a low rectal tumor. MRI pelvis shows tumor 2 mm from mesorectal fascia.

Most important implication?

- a) Low risk disease; immediate local excision
- b) Circumferential resection margin threatened
- c) No need for neoadjuvant therapy
- d) Diagnosis is benign polyp

Q20. A 70-year-old woman presents with:

- Weight loss
- Iron deficiency anemia

Colonoscopy:

Circumferential ascending colon growth.

Most common molecular pathway?

- a) APC pathway
- b) Lynch syndrome pathway
- c) Microsatellite instability pathway
- d) MEN pathway

Q21. CT Scan shows:

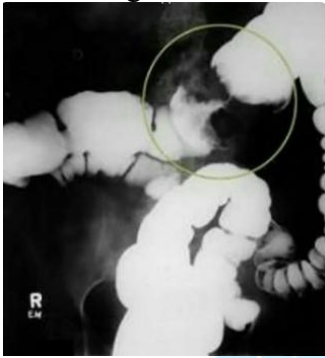
- Circumferential thickening



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

- Apple-core narrowing
- Sigmoid colon



Most likely diagnosis?

- a) Crohn disease
- b) Colorectal carcinoma
- c) Diverticulitis
- d) Ischemic colitis

Q22. A patient develops painless progressive jaundice and palpable gallbladder.

Most likely diagnosis?

- a) Gallstones
- b) Acute cholecystitis
- c) Carcinoma head of pancreas
- d) Chronic pancreatitis

Q23. A 67-year-old man presents with progressive dysphagia and weight loss.

Upper GI endoscopy:

- Mid-thoracic ulceroproliferative lesion

Biopsy:

- Squamous cell carcinoma

PET-CT:

- Multiple para-aortic lymph nodes

Most important factor rendering this lesion unresectable?

- a) Tumor length >6 cm
- b) Weight loss >10%
- c) Para-aortic nodal metastasis
- d) Squamous histology

Q24. A 70-year-old woman presents with a 3-week history of RUQ pain and weight loss. CT shows a 4 cm mass replacing the gallbladder fossa with direct invasion of liver segments IV and V, and malignant-looking porta hepatis lymph nodes. CA 19-9 is 980 U/mL. The MOST appropriate initial management is:

- a) Laparoscopic cholecystectomy with intraoperative frozen section
- b) Staging CT chest + MRI liver + staging laparoscopy before considering resection
- c) Immediate right hepatectomy
- d) Gemcitabine + cisplatin palliative chemotherapy without further staging

Q25. A 65-year-old man with a 3 cm mass at the lower CBD has biliary obstruction. CT



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

shows no distant metastases but portal vein involvement at the hepatoduodenal ligament. What does portal vein involvement indicate regarding resectability?

- a) Absolute contraindication to surgery in all cases
- b) Relative contraindication - portal vein resection and reconstruction is feasible in specialist centres with acceptable mortality
- c) No impact on resectability - portal vein involvement does not change surgical approach
- d) Requires TIPS procedure first before surgery

Q26. Which serum marker combination is MOST useful in differentiating cholangiocarcinoma from a benign biliary stricture?

- a) CEA + AFP
- b) CA 19-9 + IgG4
- c) ALP + GGT alone
- d) Bilirubin + AST

Q27. A 49-year-old man presents with recurrent abdominal pain, postprandial fullness, and repeated episodes of peptic ulcer disease despite high-dose proton pump inhibitors. Endoscopy reveals multiple ulcers in the distal duodenum and proximal jejunum. Fasting serum gastrin is 1600 pg/mL. Secretin stimulation test shows a paradoxical rise in gastrin.

Which associated syndrome should be actively screened for?

- a) MEN 1
- b) MEN 2A
- c) VHL syndrome
- d) Gardner syndrome

Q28. A 65-year-old man presents with painless total haematuria and a 5 kg weight loss. CT urogram shows a 4 cm solid mass in the right kidney. No metastases. CT characteristics: heterogeneous enhancement, cystic components (Bosniak IIF). The most appropriate management?

- a) Surveillance only - Bosniak IIF cysts are always benign
- b) Nephron-sparing surgery (partial nephrectomy) or radical nephrectomy - solid mass with haematuria requires surgical resection
- c) Percutaneous ablation (RFA/cryoablation) - first-line for all renal masses
- d) Empirical immunotherapy with sunitinib - treat as metastatic RCC

Q29. A 65-year-old smoker presents with:

- Painless gross hematuria
- No flank pain
- No dysuria

Ultrasound reveals a bladder mass.



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026



Most likely diagnosis?

- a) Renal cell carcinoma
- b) Urothelial carcinoma of bladder
- c) BPH
- d) Prostatitis

Q30. A 68-year-old man with metastatic castration-resistant prostate cancer (mCRPC) has been on ADT (leuporelin) for 24 months. His testosterone is castrate (<50 ng/dL). Despite castrate testosterone, his PSA has risen from 4.2 to 68 ng/mL over 6 months. Bone scan shows 3 new bone metastases. He has no visceral metastases. ECOG performance status 1. He is docetaxel-naïve. Which of the following is the MOST appropriate next treatment?

- a) Stop LHRH agonist - castration is no longer effective
- b) Enzalutamide OR abiraterone + prednisone - first-line options for mCRPC in docetaxel-naïve patients; continue ADT (castration maintained throughout)
- c) Bilateral orchiectomy - overcomes LHRH agonist resistance
- d) Sipuleucel-T immunotherapy - first-line for all mCRPC patients

Q31. A patient with medullary thyroid carcinoma is diagnosed.

What investigation is mandatory before surgery?

- a) Serum prolactin
- b) Serum gastrin
- c) Exclude pheochromocytoma
- d) Colonoscopy

Q32. A 68-year-old man presents with:

- Rapidly enlarging thyroid swelling
- Dysphagia
- Stridor
- Weight loss

FNAC:

- Pleomorphic giant cells

Diagnosis?

- a) Papillary carcinoma
- b) Follicular carcinoma
- c) Medullary carcinoma
- d) Anaplastic carcinoma

Q33. A 32-year-old woman presents with a thyroid swelling noticed during pregnancy. Ultrasound demonstrates:



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- 1.8 cm hypoechoic nodule
- Microcalcifications
- Irregular margins
- Taller-than-wide appearance

FNAC demonstrates:

- Nuclear grooves
- Intranuclear inclusions
- Orphan Annie nuclei

Which operation is most appropriate?

- a) Hemithyroidectomy only
- b) Near-total thyroidectomy
- c) Total thyroidectomy
- d) Radioactive iodine without surgery

Q34. A 14-year-old boy presents with acute scrotal pain of 4 hours duration. On examination, the left testis is high-riding, horizontal, and exquisitely tender. The cremasteric reflex is absent. What is the DIAGNOSIS and time window for testicular salvage?

- a) Epididymo-orchitis - IV antibiotics and scrotal support
- b) Testicular torsion - surgical exploration within 4-6 hours gives >90% salvage
- c) Torsion of hydatid of Morgagni - conservative management
- d) Incarcerated inguinal hernia - emergency hernia repair

Q35. A 64-year-old woman presents with:

- Recurrent cholangitis
- Jaundice

MRCP demonstrates:

- Fusiform CBD dilatation

Diagnosis?

- a) Todani Type I Choledochal cyst
- b) Type II
- c) Type III
- d) Type IV

Q36. Contrast CT demonstrates:

- Target sign
- Bowel within bowel appearance



Diagnosis?

- a) Volvulus
- b) Intussusception



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

- c) Crohn disease
- d) Sigmoid carcinoma

Q37. A 24-year-old man presents with recurrent episodes of abdominal pain and intermittent bleeding per rectum.

Technetium-99m scan demonstrates ectopic gastric mucosa.



Most likely diagnosis?

- a) Duplication cyst
- b) Meckel diverticulum
- c) Crohn disease
- d) Jejunal diverticulum

Q38. A 6-month-old boy has bilateral undescended testes (cryptorchidism). Both testes are palpable at the external inguinal ring. What is the recommended AGE for orchidopexy?

- a) Age 6 months - immediate surgery to prevent torsion
- b) Age 6-12 months - surgery recommended; latest by 18 months to optimise fertility and reduce malignancy risk
- c) Puberty - testes often descend spontaneously
- d) Age 5 years - standard wait-and-see approach

Q39. A 55-year-old man with a GCS of 6 (E1V2M3) is intubated and sedated in the ICU following a traumatic brain injury. After 5 days of sedation, the team needs to assess his neurological progress. Sedation is held and he is allowed to wake up. His sedation hold assessment shows: eyes open spontaneously (E4), he is intubated so verbal response cannot be assessed, and he obeys commands with his right hand (M6). What is his GCS, how is the verbal component documented for an intubated patient, and what does his motor score of 6 indicate about prognosis?

- a) GCS cannot be calculated in intubated patients - use FOUR score instead
- b) GCS = E4 + V1 (none - cannot speak) + M6 = 11T; verbal component documented as "T" (tube/intubated); M6 (obeys commands) is the most favourable motor response and indicates significant neurological recovery
- c) GCS = E4 + V5 (assume oriented) + M6 = 15 - patient has fully recovered
- d) GCS verbal = 3 (inappropriate words assumed) for all intubated patients

Q40. A 58-year-old hypertensive smoker presents with sudden onset severe tearing pain radiating from chest to back. CXR shows widened mediastinum. CT aorta shows dissection of the ascending aorta (Stanford Type A). What is the IMMEDIATE management?



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- a) IV antihypertensive therapy (labetalol) and monitoring in ICU - Type A managed medically
- b) Emergency surgical repair of ascending aorta - Type A is a surgical emergency
- c) TEVAR (Thoracic endovascular aortic repair) - minimally invasive option for Type A
- d) Thrombolysis - aortic dissection may be caused by thrombosis

Q41. A 60-year-old man presents with sudden onset of severe tearing abdominal and back pain. He is hypotensive (BP 80/60). He has a history of hypertension. On palpation, there is a pulsatile, expansile epigastric mass. What is the SINGLE MOST IMPORTANT next step?

- a) CT angiography of the aorta
- b) Activate the ruptured AAA protocol - immediate transfer to theatre, permissive hypotension, do NOT wait for CT
- c) IV fluid resuscitation to MAP >65 mmHg
- d) Urgent echocardiogram to exclude aortic dissection

Q42. A 50-year-old man undergoes elective right hemicolectomy. On post-operative day 7, he develops swollen, painful, erythematous right calf with a D-dimer of 4.2 µg/mL. Wells score = 3 (high probability). What is the management?

- a) Repeat D-dimer in 2 weeks
- b) Doppler USS of the right leg - if DVT confirmed: therapeutic LMWH (enoxaparin 1.5 mg/kg OD) or DOAC for 3 months
- c) CTPA immediately - suspected PE
- d) IVC filter insertion - post-surgical DVT always requires filter

Q43. A 55-year-old man presents with a 6 cm hard, well-defined lump in the right parotid region. FNA shows oncocytic cells with admixed lymphocytes in a lymphoid background. He is a smoker. Bilateral enlargement is noted. What is the diagnosis?

- a) Pleomorphic adenoma
- b) Warthin's tumour (papillary cystadenoma lymphomatosum) - bilateral in 10%, associated with smoking, oncocytic cells + lymphoid stroma
- c) Mucoepidermoid carcinoma
- d) Lymphoma

Q44. A 50-year-old man with MEN2A is found to have bilateral adrenal pheochromocytoma on MRI. He also has a 2 cm right thyroid nodule (medullary carcinoma confirmed on FNA). In what ORDER should these conditions be treated?

- a) Thyroidectomy first - medullary carcinoma is the primary malignancy
- b) Pheochromocytoma resection first (bilateral adrenalectomy) - hypertensive crisis during thyroid surgery fatal; then thyroidectomy
- c) Both simultaneously in one operation
- d) Medical management only - MEN2A patients are poor surgical candidates

Q45. A 55-year-old man presents with episodic hypertension (BP 220/130), sweating, palpitations, and headaches. 24h urinary metanephrines are 4× upper limit of normal. CT adrenals shows a 4 cm right adrenal mass. Pre-operative management before adrenalectomy?

- a) Beta-blocker (propranolol) first - to control hypertension
- b) Alpha-blocker (phenoxybenzamine) first for 2 weeks, then add beta-blocker - essential to prevent hypertensive crisis



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

- c) ACE inhibitor for blood pressure control
d) Emergency adrenalectomy without pre-operative preparation
- Q46. A 32-year-old woman presents with 3 months of amenorrhea, severe lower abdominal pain, and dizziness. Urine pregnancy test is positive. Transvaginal ultrasonography shows an empty uterine cavity, a complex adnexal mass, and significant free fluid in the pouch of Douglas. On examination, blood pressure is 90/60 mmHg, pulse rate is 110/min, and cervical motion tenderness is present. What is the most likely diagnosis?
- a) Missed abortion
b) Incomplete abortion
c) Ruptured ectopic pregnancy
d) Hydatidiform mole
- Q47. A 14-year-old girl presents with severe cyclic lower abdominal pain occurring every month for the last six months. She has developed normal breasts and pubic hair but has never menstruated. Examination reveals a tender pelvic mass and a bluish bulging membrane at the vaginal introitus. Pelvic ultrasound demonstrates hematocolpos and hematometra. Which embryological defect is responsible for this condition?
- a) Failure of Müllerian duct fusion
b) Failure of resorption of uterine septum
c) Failure of canalization of vaginal plate
d) Agenesis of paramesonephric ducts
- Q48. A 69-year-old man presents with urinary hesitancy, weak stream, nocturia, and diffuse low back pain. Digital rectal examination reveals a hard irregular prostate. PSA is markedly elevated, and bone scan shows multiple osteoblastic lesions. He is started on a monthly depot injection. Ten days later, he develops worsening bone pain and urinary symptoms, which subsequently improve after one month. What is the mechanism responsible for the initial worsening?
- a) Competitive inhibition of androgen receptors
b) Continuous stimulation of pituitary GnRH receptors
c) Inhibition of 5-alpha reductase
d) Suppression of adrenal androgen synthesis
- Q49. A 38-year-old woman seeks counseling regarding ovarian cancer prevention. She is BRCA-negative but has a mother who developed epithelial ovarian carcinoma at age 62 years. She has completed two pregnancies and desires a non-surgical strategy to reduce future risk. Which intervention has the strongest evidence for reducing epithelial ovarian cancer risk?
- a) Annual transvaginal ultrasonography
b) Routine CA-125 monitoring
c) Prophylactic bilateral oophorectomy at age 50
d) Long-term combined oral contraceptive use
- Q50. A term pregnant woman is undergoing continuous fetal monitoring during labor. Which of the following conditions is least likely to cause fetal bradycardia?
- a) Placental abruption
b) Fetal head compression



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

- c) Congenital heart disease
- d) Maternal fever

Q51. A 65-year-old postmenopausal woman presents with progressive abdominal distension, ascites, and weight loss. Imaging reveals a complex ovarian mass with extensive omental deposits. Serum CA-125 level is 990 U/mL. Which is the most likely diagnosis?

- a) Granulosa cell tumor
- b) Endometrioid carcinoma
- c) Mucinous cystadenocarcinoma
- d) Serous cystadenocarcinoma

Q52. A 45-year-old woman presents with abnormal uterine bleeding. Transvaginal ultrasonography demonstrates an endometrial thickness of 15 mm. What is the most appropriate next step in management?

- a) MRI pelvis
- b) Hysterectomy
- c) Endometrial biopsy
- d) Hysteroscopy with fractional curettage

Q53. A 18-year-old girl was brought by her mother to the outpatient department with complaint of primary amenorrhea. Her breast development correspond to tanner stage 5. Which of the following finding will clinch the diagnosis of MRKH syndrome?

- a) Absent axillary and pubic hairs
- b) Suprapubic bulge on per abdominal examination
- c) Absence of uterus, cervix and vagina on ultrasound investigation
- d) Galactorrhea

Q54. All of the following signs are seen by 8 weeks pregnancy except:

- a) Goodell sign
- b) Lightening
- c) Chadwick sign
- d) Hegar sign

Q55. All of the following are correct about pregnancy except:

- a) Increase in GFR
- b) Left axis deviation in ECG
- c) Increase in coagulation factor 11
- d) Increase in oxygen carrying capacity

Q56. A 37-year-old lady presents at 18 weeks gestation with dilated cervix. Cervical length is 14 MM. She is insisting for cervical cerclage. All of the following statements are correct except:

- a) Cerclage may be done upto 24 weeks
- b) Best cerclage is MC Donald stitches
- c) Ruptured membranes are contraindications for cerclage
- d) Most common time of abortion due to cervical incompetence is 12-14 weeks gestation



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

- Q57. A lady presents at 28 weeks gestation with profuse vaginal bleeding. All of the following statements appear correct regarding this except?
- Placental implantation at internal OS may cause this
 - Abdomen will be hard and tender in abruptio placentae
 - Management of placenta praevia is caesarean section at 37 weeks if vitals are stable
 - Couvelaire uterus may be seen in placenta praevia
- Q58. Which of the following is not included in quadruple test?
- Alphafetoprotein
 - Inhibin B
 - hCG
 - Unconjugated estriol
- Q59. All of the following statements are correct except:
- Ideal time to check nuchal translucency by USG is 11-13 weeks pregnancy
 - Cardiotocography is not included in biophysical profile scoring
 - ABO incompatibility may cause nonimmune hydrops fetalis
 - Tense but nontender abdomen with increased symphysiofundal height may be due to polyhydramnios
- Q60. All of the following statements are correct except:
- Infraumbilical flattening with FHS heard on sides may indicate occipitoposterior position
 - Studdiford criteria is used for abdominal ectopic pregnancy
 - Maximum dose of PGF2@ to treat postpartum haemorrhage is 2 MG
 - Footling breech is safest breech
- Q61. A lady presents at 36 weeks gestation with complaint of feeling lightheadedness and dizziness when she lies on her back. She is alright when she lies on her side or walks. Which of the following statements is false?
- Supine hypertension may predict pregnancy induced hypertension
 - Persistence of diastolic notch in uterine artery after 22 weeks gestation may also predict PIH
 - DOC to treat PIH in asthmatic lady is nifedipine
 - Earliest evidence of magnesium sulphate toxicity is tonic clonic seizures
- Q62. All of the following are severe features in preeclampsia except:
- Headache
 - Epigastric pain
 - Blurred vision
 - Blood pressure 150/100 MM HG at 16 weeks pregnancy
- Q63. All of the following statements are correct except:
- In modern obstetrics, best test for monitoring sensitized Rh negative mother is fetal middle cerebral artery color Doppler
 - Congenital malformations are seen in gestational diabetes mellitus
 - Warfarin is stopped at 36 weeks pregnancy and LMWH is started if she is on anticoagulation
 - Management of a lady in labor with active herpetic lesions on vulva is acyclovir and elective caesarean section



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

Q64. All of the following statements are correct except:

- a) Carboprost is contraindicated in bronchial Asthama
- b) Indomethacin if continued beyond 32 weeks pregnancy may cause premature closure of ductus arteriosus
- c) Magnesium sulphate has neuroprotective role in preterm labor
- d) Methylergometrine is DOC to treat PPH in hypertensive lady

Q65. All of the following statements are correct about septate uterus except:

- a) It is most common congenital malformation of uterus
- b) Its most common complication is abortion
- c) Its treatment is hysteroscopic septoplasty
- d) It is commonly associated with ectopic pregnancy

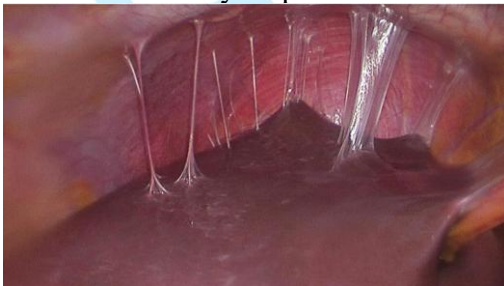
Q66. All of the following statements are correct except:

- a) Plasma glucose assessment should be done in first antenatal visit
- b) According to dipsi criteria, overt diabetes is diagnosed when blood glucose is more than 200 mg %
- c) First step in shoulder dystocia management is suprapubic pressure
- d) Most specific malformation of diabetes in pregnancy is caudal regression syndrome

Q67. All of the following statements are correct about morbidly adherent placenta except:

- a) USG and MRI may be used for diagnosis
- b) Most important risk factor is placenta praevia in current pregnancy with history of previous lower segment caesarean section
- c) Management is induction of labor at 37 weeks
- d) Most common variety is placenta accreta

Q68. A 30-year-old woman with recurrent pelvic inflammatory disease presents with fever and severe right upper quadrant pain. Diagnostic laparoscopy demonstrates thin “violin-string” adhesions between the liver capsule and anterior abdominal wall. Which organism is most commonly responsible?



- a) *Treponema pallidum*
- b) *Haemophilus ducreyi*
- c) *Neisseria meningitidis*
- d) *Chlamydia trachomatis*

Q69. A 16-year-old girl presents with primary amenorrhea. She has normal breast development but sparse pubic and axillary hair. Pelvic ultrasonography shows absence of the uterus and upper vagina. Karyotyping reveals a 46,XY genotype. Which of the following is the



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

most likely diagnosis?

- a) Müllerian agenesis (MRKH syndrome)
- b) Turner syndrome
- c) Complete androgen insensitivity syndrome
- d) Pure gonadal dysgenesis

Q70. A 48-year-old woman is scheduled for laparoscopic hysterectomy for symptomatic fibroids. During counseling, she asks about disadvantages of laparoscopic surgery compared with open abdominal hysterectomy. Which of the following is a recognized disadvantage?

- a) Increased postoperative pain
- b) Increased blood loss
- c) Prolonged recovery period
- d) Increased risk of thermal injury

Q71. A 28-year-old woman with a history of multiple sexual partners presents with chronic pelvic pain, intermenstrual bleeding, and mucopurulent cervical discharge. Pelvic examination reveals cervical motion tenderness. Which organism is the most likely cause?

- a) Candida albicans
- b) Herpes simplex virus
- c) Neisseria gonorrhoeae
- d) Mycobacterium tuberculosis

Q72. A 30-year-old woman presents 6 months after vaginal delivery with persistent pain at the site of episiotomy repair. The pain worsens during menstruation. Examination reveals a tender nodular lesion at the scar site. What is the most likely diagnosis?

- a) Inclusion cyst
- b) Scar endometriosis
- c) Retained suture granuloma
- d) Vulval melanoma

Q73. A 29-year-old G3P2 woman presents at 9 weeks gestation. She underwent cervical conization one year ago for CIN III and has had normal follow-up Pap smears. What is the most appropriate management during this pregnancy?

- a) Immediate cervical cerclage
- b) Ultrasound assessment of cervical length
- c) Strict bed rest
- d) Elective abdominal cerclage

Q74. A 16-year-old girl presents with progressively painful menstrual periods. Menarche occurred at age 13 years. Cycles are regular. Pain begins shortly before menstruation and lasts for 1-2 days. She also experiences nausea, diarrhea, and fatigue during menses. Pelvic examination is normal. What is the most likely diagnosis?

- a) Endometriosis
- b) Adenomyosis
- c) Somatic symptom disorder
- d) Primary dysmenorrhea



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- Q75. A 29-year-old woman at 38 weeks gestation presents with sudden severe abdominal pain, vaginal bleeding, hypotension, and fetal bradycardia. She previously underwent a classical cesarean section at 24 weeks gestation. Abdominal examination reveals loss of uterine contour and palpable fetal parts. Which risk factor most likely contributed to this condition?
- a) First-trimester bleeding
 - b) Cocaine use
 - c) Short interpregnancy interval
 - d) Previous classical cesarean section

- Q76. A 30-year-old pregnant woman with a history of autoimmune disease delivers a newborn who develops an erythematous annular rash over the face and scalp within the first few weeks of life. The infant is otherwise healthy, but ECG reveals congenital complete heart block. The pediatrician suspects neonatal lupus erythematosus caused by transplacental passage of maternal autoantibodies.

Which of the following antibodies is most commonly associated with this condition?

- a) Anti-double-stranded DNA (Anti-dsDNA) antibody
- b) Anti-Smith (Anti-Sm) antibody
- c) Anti-Ro (SS-A) antibody
- d) Anti-histone antibody

- Q77. A 43-year-old woman has symmetric small-joint pain, morning stiffness >1 hour, and positive anti-CCP. X-ray: periarticular osteopenia & marginal erosions. What is the BEST early treatment strategy?

- a) NSAIDs only
- b) Start Methotrexate
- c) Hydroxychloroquine monotherapy
- d) Oral steroids long-term

- Q78. A 67-year-old man presents with chronic joint pain in the midfoot and knee. Exam shows bony irregularities and decreased range of motion. X-ray shows “punched-out” erosions with overhanging edges. He has firm, yellowish nodules over his elbows. What is the diagnosis?

- a) Pseudogout
- b) Chronic tophaceous gout
- c) Rheumatoid arthritis
- d) Septic arthritis

- Q79. A 24-year-old man is evaluated for lifelong mild jaundice that worsens during fasting or illness. He has no abdominal pain, pruritus, or weight loss. Physical examination is unremarkable other than scleral icterus. Liver is smooth and normal in color

Laboratory results:

Total bilirubin: 3.8 mg/dL

Direct (conjugated) bilirubin: elevated

AST/ALT: normal

ALP: normal

Urine coproporphyrin analysis shows increased type I coproporphyrin.

Ultrasound of the liver is normal.



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MOCK TEST – 7 PAPER -2 TEST DATED ON 18 JUNE 2026

Which diagnosis best explains his presentation?

- a) Rotor syndrome
- b) Dubin-Johnson syndrome
- c) Gilbert syndrome
- d) Autoimmune hepatitis

Q80. A 33-year-old woman presents with malaise and mild jaundice. She reports flu-like symptoms 3 weeks earlier.

Hepatitis B serology:

HBsAg: Negative

Anti-HBs: Negative

Anti-HBc IgM: Positive

HBeAg: Negative

What is the most likely explanation for this pattern?

- a) Past resolved infection
- b) Vaccinated, no infection
- c) Window period of acute HBV
- d) Chronic inactive carrier state

Q81. A 34-year-old man with a 7-year history of ulcerative colitis presents with progressive fatigue, pruritus, and intermittent jaundice. Laboratory investigations reveal elevated alkaline phosphatase (ALP) and γ -glutamyl transferase (GGT). Ultrasound of the abdomen shows no gallstones or extrahepatic biliary obstruction.

Which of the following is the investigation of choice for diagnosing this condition?

- a) Liver biopsy
- b) Endoscopic ultrasound (EUS)
- c) Magnetic resonance cholangiopancreatography (MRCP)
- d) Contrast-enhanced CT abdomen

Q82. A 52-year-old man presents with progressive fatigue, decreased libido, and intermittent pain in the second and third MCP joints of both hands. He reports that his "skin seems to be getting darker," even though he has not been spending much time in the sun. Over the last year, he also developed poor glycemic control, requiring escalation of diabetes medications.

He denies alcohol use and has no history of blood transfusions. On examination, his skin appears bronze-colored, and he has hepatomegaly without tenderness. Testicular size is slightly reduced.

Laboratory results show:

AST: 92 U/L

ALT: 88 U/L

Fasting glucose: elevated

Serum ferritin: markedly elevated

Transferrin saturation: very high ($\approx 70\%$)

HbA1c: elevated

Genetic testing reveals C282Y homozygosity in the HFE gene.

Which diagnosis best fits this presentation?

- a) Wilson disease
- b) Alcoholic liver disease



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- c) Hereditary hemochromatosis
- d) Primary biliary cholangitis

Q83. A 17-year-old boy is brought to the clinic for evaluation of persistent hypertension detected during a school health check-up. He complains of occasional headaches and muscle weakness. His father was diagnosed with hypertension at a young age. On examination, blood pressure is 170/100 mmHg.

Laboratory investigations reveal:

Serum potassium: 2.8 mEq/L

Serum bicarbonate: 31 mEq/L

Plasma renin activity: low

Serum aldosterone: low

CT scan of the adrenal glands is normal. Which of the following is the most likely diagnosis?

- a) Primary hyperaldosteronism (Conn syndrome)
- b) Liddle syndrome
- c) Addison disease
- d) Apparent mineralocorticoid excess due to licorice ingestion

Q84. A 72-year-old man is brought to the emergency department with profuse vomiting and diarrhea for the past 4 days. He complains of dizziness, weakness, and decreased urine output. On examination, he is dehydrated with dry mucous membranes and postural hypotension. Laboratory investigations reveal elevated serum creatinine and blood urea nitrogen. Urinalysis shows a bland sediment without casts.

The physician suspects prerenal acute kidney injury (AKI) secondary to volume depletion. Which of the following combinations of laboratory findings is most consistent with prerenal AKI?

- a) BUN:Cr ratio >20:1, FENa <1%, Urine Na⁺ <20 mEq/L
- b) BUN:Cr ratio <15:1, FENa >2%, Urine Na⁺ >40 mEq/L
- c) BUN:Cr ratio >20:1, FENa >2%, Urine Na⁺ >40 mEq/L
- d) BUN:Cr ratio <15:1, FENa <1%, Urine Na⁺ <20 mEq/L

Q85. A 58-year-old man with a 20-year history of poorly controlled type 2 diabetes mellitus presents with progressive swelling of both feet and frothy urine. Laboratory investigations reveal:

Urine albumin: 3+

24-hour urinary protein: 3.8 g/day

Serum creatinine: 2.1 mg/dL

HbA1c: 10.2%

Fundus examination shows diabetic retinopathy. A renal biopsy is performed to evaluate the cause of his chronic kidney disease.

Which of the following microscopic findings is most characteristic of diabetic nephropathy?

- a) Crescent formation in Bowman's space
- b) Kimmelstiel-Wilson nodules (nodular glomerulosclerosis)
- c) Wire-loop lesions due to subendothelial immune deposits
- d) Podocyte foot process effacement with normal light microscopy

Q86. A 34-year-old woman is brought to the clinic with recurrent episodes of sweating, palpitations, tremors, and confusion, especially in the early morning and when she misses



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meals. These symptoms are relieved immediately after eating food or drinking glucose-containing solutions.

During one episode in the clinic, her blood glucose is measured and found to be 38 mg/dL, and symptoms resolve after intravenous dextrose administration.

Which of the following best describes this clinical presentation?

- a) Cushing's triad
- b) Whipple's triad
- c) Reynolds pentad
- d) Beck's triad

Q87. A 29-year-old woman presents with progressive fatigue, weight loss, and darkening of skin over the past 4 months. She also reports dizziness on standing and salt cravings. On examination, she has low blood pressure (90/60 mmHg) and hyperpigmented palmar creases.

Laboratory investigations show:

Hyponatremia

Hyperkalemia

Low serum cortisol

Elevated plasma ACTH

Which of the following is the most likely diagnosis?

- a) Cushing syndrome
- b) Primary adrenal insufficiency (Addison disease)
- c) Secondary adrenal insufficiency
- d) Conn syndrome

Q88. A 26-year-old woman presents with excessive thirst and passage of large volumes of dilute urine for the past 3 weeks. She also reports waking up multiple times at night to drink water and urinate. There is no history of diabetes mellitus or diuretic use. On examination, she appears dehydrated.

Laboratory investigations reveal:

Serum sodium: high-normal

Serum osmolality: high

Urine osmolality: low

Urine specific gravity: 1.002

After administration of desmopressin, there is a marked increase in urine osmolality and a decrease in urine volume. Which of the following is the most likely diagnosis?

- a) Nephrogenic diabetes insipidus
- b) Primary polydipsia
- c) Central diabetes insipidus
- d) SIADH

Q89. A young male presents with joint pains and backache with relief of symptoms on movement/exercise. The most likely diagnosis is?

- a) Rheumatoid arthritis
- b) Ankylosing spondylitis
- c) Poly-articular juvenile arthritis
- d) Psoriatic arthropathy



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- Q90. A 55-year-old chronic alcoholic presents with increasing fatigue, weakness, anorexia, and jaundice over the past several months. Physical examination finds mild ascites and gynecomastia. A liver biopsy reveals regenerative nodules of hepatocytes surrounded by fibrosis. Which of the following is the source of the excess collagen deposited in the fibrotic bands?
- Hepatocytes
 - Kupffer cells
 - Ito cells
 - Endothelial cells
- Q91. A 49-year-old woman presents with increasing fatigue and is found to have elevated liver enzymes (AST and ALT).. All serologic tests for viral markers are within normal limits. A liver biopsy reveals interface hepatitis. There are no granulomas present, and there is no evidence of fibrosis surrounding any of the bile ducts within the portal triads. Anti-smooth-muscle antibodies and antinuclear antibodies are found in the patient's serum. Which of the following is the most likely diagnosis?
- Autoimmune hepatitis
 - Chronic persistent hepatitis
 - Primary biliary cirrhosis
 - Primary sclerosing cholangitis
- Q92. A significant reduction in systemic arterial pressure is characteristically observed during inspiration occurs in all the following conditions except:
- Severe asthma attack
 - Aortic dissection causing aortic regurgitation and cardiac tamponade
 - Massive pulmonary embolism
 - Massive haemorrhage
- Q93. Which of the heart valve is most likely to be involved by infective endocarditis following aseptic abortion
- Aortic valve
 - Tricuspid valve
 - Pulmonary valve
 - Mitral valve
- Q94. Which one of the following features is least likely to occur in a patient with visceral leishmaniasis :
- Massive splenomegaly
 - Diarrhea
 - Pyrexia
 - Pancytopenia
- Q95. A patient presents with chronic small bowel diarrhoea. Duodenal biopsy shows villous atrophy. Anti-endomysial antibodies and IgA tTG antibodies are positive. What is the treatment of choice?
- Gluten-free diet
 - Antibiotics



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- c) Loperamide
- d) 5-ASA

Q96. An 18 yrs old male presents with massive hematemesis: he has history of fever for the past 14 days for while he was managed with drugs: modrate splenomegaly is present: diagnosis is:

- a) NSAID induced duodenal ulcer
- b) Drug induced gastritis
- c) Esophageal varices
- d) None of the above

Q97. You have recommended an X drug for symptoms like Polyarticular arthropathy with morning stiffness of joints for more than one hour. Now Patient has diarrhea along with low haemoglobin Which is the likely drug?

- a) Methotrexate
- b) Sulfasalazine
- c) Leflunamide
- d) Hydroxychloroquine

Q98. A 4 year old child presents complaining of bilateral lower limb edema. Examination showed blood pressure 130/80 mmHg. Investigations: serum creatinine :1.0 mg/dl, urine analysis: 2+ protein, hyaline casts. Renal biopsy showed picture of minimal change disease. What is the most possible etiological diagnosis that should be looked for?

- a) Chronic Hepatitis
- b) Drug induced
- c) Lupus nephritis
- d) Lymphoma

Q99. 27-year-old male presents with malaise, pyrexia, lymphadenopathy and a maculopapular rash. The Monospot test is negative. Given a history of high risk sexual behaviour you are asked to exclude a HIV seroconversion illness. The most appropriate investigation is :

- a) Antibodies to HN-2
- b) gp 120 polymerase chain reaction
- c) p24 antigen test
- d) Antibodies to HN-I

Q100. An 18 year old male presents with features of infectious mononucleosis. However, he has now noticed marked swelling in his face and legs. Blood reveal a low albumin. Urinalysis reveals proteinuria and 24-hour collection reveal a protein excretion of 4g/day. Renal biopsy is normal. What is the most likely underlying cause of his symptoms?

- a) Minimal change disease leading to nephrotic syndrome
- b) Focal segmental glomerulosclerosis
- c) Minimal change disease leading to nephritic syndrome
- d) Membranous nephropathy

Q101. A 70 year old patient with a bed ridden status from last 3 months brought to the



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emergency with the c/o sudden shortness of breath, increased respiratory rate His ECG showed sinus tachycardia with S1Q3T3 pattern Which of the following is the likely diagnosis?

- a) ASTEMI
- b) NSTEMI
- c) CHF
- d) Pulmonary embolism

Q102. A 30 year-old male with no previous history presents with a syncopal attack and is found to have a pulse of 110/70, a slow rising carotid pulse, a thrusting hyperactive left ventricular apex beat and an ejection systolic murmur with a diminished second sound. The most likely diagnosis is:

- a) 60 Hypertrophic cardiomyopathy
- b) Pulmonic stenosis
- c) Aortic Stenosis
- d) Ventricular septal defect

Q103. A 35 year old lady has had no periods for two years. She also notices increased hair growth. Examination reveals male pattern balding and hair growth, and clitoromegaly. The likely diagnosis:

- a) Cushing's disease
- b) Adrenal tumour
- c) MEN
- d) Congenital adrenal hyperplasia

Q104. Which chromosomal aberration is seen in Turner's syndrome?

- a) 47, XXY
- b) 45, XO
- c) 46, XY
- d) 46, XX

Q105. Most common cardiomyopathy associated with alcoholism is:

- a) Hypertrophic
- b) Dilated
- c) Ischemic
- d) All of the above

Q106. A child is being assessed in a neurodevelopmental clinic. While evaluating the different milestones, which from among the options is likely to be the first to have developed?

- a) Recognizing mother
- b) Babbling
- c) Sitting without support
- d) Pincer grasp

Q107. An 11-month old infant has history of developmental regression, seizures and fine sparse typically steely hair. Hair biopsy revealed twists along its axes and fragmentation at regular intervals. These may be features of which of these deficiencies?

- a) Zinc deficiency



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- b) Copper deficiency
- c) Iron deficiency
- d) Vitamin C deficiency

Q108. Which of these may be considered a good marker of “severe wasting” in children?

- a) Head circumference
- b) Mid-arm circumference
- c) Weight for height
- d) Height for age

Q109. A newborn baby after delivery by a cesarian section develops a soft swelling on the scalp about 6 hours after birth. The swelling is in the center, and appears soft, boggy, and fluctuant (shown in image). What is your diagnosis?



- a) Sudural hematoma
- b) Subgaleal hemorrhage
- c) Cephalhematoma
- d) Caput secundum

Q110. All of the following childhood seizures can be effectively managed with sodium valproate except

- a) Generalized tonic clonic seizures
- b) Infantile spasms
- c) Absence seizures
- d) Juvenile myoclonic epilepsy

Q111. A mother brings her 7-year-old child to you complaining of frequent episodes of bed wetting by the child. The child had never achieved consistent day- or nighttime dryness since birth. Remaining developmental milestones were appropriate for age. What will be your treatment of choice?

- a) Start oxybutynin treatment
- b) Bell and alarm treatment
- c) Psychological counselling sessions
- d) Start desmopressin treatment

Q112. A 29-week preterm neonate with history of perinatal asphyxia is being resuscitated after birth. When initiating PPV with bag and mask, what is the optimum oxygen concentration advisable to be used in this neonate?



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- a) 21%
- b) 21-30%
- c) 30-40%
- d) 40-70%

Q113. All of the following are known common causes of neonatal meningitis except:

- a) S. aureus
- b) H. influenza
- c) GBS
- d) Klebsiella

Q114. An infant is being evaluated for congenital malformations. His mother had history of poorly-controlled type 2 diabetes at the time of delivery. Which of these complications is most specific to maternal diabetes?

- a) Congenital heart defects
- b) Congenital renal anomalies
- c) Neural tube defects
- d) Sacral agenesis

Q115. A newborn developed rapid eyelid blinking movement with intermittent fluttering immediately after birth. Rhythmic jerking of left lower limb is also noticed. He is diagnosed having a seizure. Which of these is expected to be the most likely cause?

- a) Hypoxia
- b) Hypoglycemia
- c) Infections
- d) Trauma

Q116. Which of these is considered the most common type of neonatal seizures?

- a) Myoclonic
- b) Focal
- c) Subtle
- d) Absence

Q117. What is the most common time of disappearance of the primitive neonatal reflex shown in the image?



- a) 1 month
- b) 3 months
- c) 6 months
- d) 9 months



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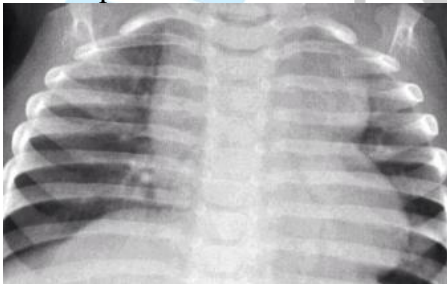
Q118. A child with severe malnutrition is brought to the health center and admitted. Treatment plan is drawn and the calorie count of ready to use therapeutic food calculated. In the first 48 hours of stabilization phase he should be started on a specialized milk-based formula that provides

- a) 75 kcal per 100 ml
- b) 150 kcal per 100 ml
- c) 200 kcal per 100 ml
- d) 250 kcal per 100 ml

Q119. All of the following drugs can be used for medical closure of an open ductus arteriosus after birth except

- a) Ibuprofen
- b) Celecoxib
- c) Paracetamol
- d) Indomethacin

Q120. Identify the congenital heart disease from the typical X ray picture that it usually presents with?



- a) PDA
- b) TOF
- c) TAPVR
- d) TGA

Q121. A 6-year-old child presents with bowing of the legs and short stature. Despite adequate vitamin D supplementation for several months, there is no clinical improvement. Investigations reveal: serum calcium 9.2 mg/dL, serum phosphate 2 mg/dL, elevated alkaline phosphatase, normal PTH, and normal arterial blood gas analysis. What is the most likely diagnosis?

- a) Nutritional rickets
- b) Vitamin D-dependent rickets type I
- c) Vitamin D-dependent rickets type II
- d) Hypophosphatemic rickets

Q122. A 68-year-old postmenopausal woman presents with back pain and a history of a low-impact vertebral fracture. Laboratory investigations reveal normal serum calcium, normal serum phosphate, and normal alkaline phosphatase levels. What is the most likely diagnosis?

- a) Osteomalacia
- b) Hyperparathyroidism
- c) Osteoporosis
- d) Paget disease of bone

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Q123. A 5-year-old boy is brought to the emergency department after falling on an outstretched hand while playing. X-ray reveals an extension-type supracondylar fracture of the humerus. Which of the following vascular structures is most commonly injured in this condition?



- a) Radial artery
- b) Brachial artery
- c) Ulnar artery
- d) Basilic vein

Q124. A 35-year-old man presents with pain and swelling over the middle part of his leg. He gives a history of a road traffic accident at the same site 2 years ago, for which he underwent treatment. X-ray of the tibia shows a dense fragment of dead bone surrounded by new bone formation (sequestrum and involucrum). What is the most likely diagnosis?



- a) Osteogenic sarcoma
- b) Ewing sarcoma
- c) Chronic osteomyelitis
- d) Multiple myeloma

Q125. A 4-year-old child is brought to the outpatient department for short stature. On examination, the child has short limbs with a relatively normal trunk, frontal bossing, and a depressed nasal bridge. Intelligence is normal. X-ray shows shortening of the proximal segments of the limbs (rhizomelia). What is the most likely diagnosis?



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- a) Osteogenesis imperfecta
- b) Achondroplasia
- c) Marfan syndrome
- d) Hypothyroidism

Q126. Pigmented macules over palm seen on clinical examination. Histologically, proliferating melanocytes are found at the dermoepidermal junction. Diagnosis?



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- a) Halo nevus
- b) Junctional nevus
- c) Dermal Melanocytic nevus
- d) Dysplastic nevus

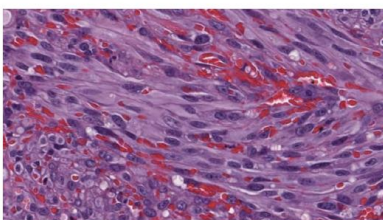
Q127. Which of the following about mycosis fungoides is not true?

- a) It is the most common skin lymphomas
- b) Pautrier's microabscess are common
- c) It has an indolent course with good prognosis
- d) It presents with diffuse erythroderma

Q128. Patient with: pruritic lesion over Left shoulder showing cigarette paper atrophy and poikiloderma with generalized lymphadenopathy. Histopathology examination of lesion shows CD4 positive Sézary Lutzner cells. What is the dermoepidermal manifestation of this disease?

- a) Discharging sinus
- b) Pin point ulcers
- c) Miliaria
- d) Pautrier's microabscess

Q129. A Retro positive patient developed erythematous plaque over Ankle sole of foot, The HPE shows-abnormal vascular endothelial cells with a typical spindle cells.



- a) Hemangioma
- b) Kaposi's sarcoma
- c) Reticulum cell carcinoma
- d) Basal cell carcinoma



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Q130. A child with fever presents with multiple tender erythematous skin lesions. On microscopic examination, the skin lesions have neutrophilic Infiltration in the dermis. What is the diagnosis?:



- a) Sweet's syndrome
- b) Behcet's syndrome
- c) Pyoderma gangrenosum
- d) Juvenile dermatosis

Q131. Idiot Savant syndrome is most commonly associated with ?

- a) ADHD
- b) Mental Retardation
- c) Rett's syndrome
- d) Autism

Q132. All are true about narcolepsy except?

- a) Familial with mutation of DQB1-MHC II antigen
- b) Sudden loss of muscle tone
- c) Irresistible desire to sleep
- d) Sudden NREM sleep

Q133. S.C.O.F.F. criteria is used for screening of ?

- a) Organic mental disorder
- b) Delirium
- c) Eating Disorder
- d) Catatonia

Q134. A women report to her physician that she can achieve orgasm only when recalling a previous, abusive boyfriend. Suspecting the presence of a paraphilia, the psychiatrist should explore for further evidence of ?

- a) Sadism
- b) Masochism
- c) Transvestism
- d) Voyeurism

Q135. Maximum duration for which a person can take himself voluntary admission under the mental health care act 2017?



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- a) 14 days
- b) 45 days
- c) 90 days
- d) 30 days

Q136. The abnormality shown in the below image can be seen in the following conditions?



- a) Jejunal atresia
- b) Hypertrophic Pyloric stenosis
- c) Ileal Atresia
- d) Annular Pancreas

Q137. The most sensitive imaging modality for detection of acute ureteric colic and ureteric stones is:

- a) X-ray KUB
- b) Ultrasound abdomen
- c) Non-contrast CT KUB (NCCT)
- d) Contrast-enhanced CT urogram

Q138. What is the name of the investigation?



- a) CT Scan
- b) PET Scan
- c) Bone Scan
- d) DEXA Scan

Q139. In a 14 yr. child with back pain, X-ray spine has been done and shown below. What is the likely diagnosis?



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- a) T.B.
- b) Langerhans Cell Histiocytosis
- c) Hemolytic Anemia
- d) Traumatic Vertebral Collapse

Q140. 3 year old child left unattended has presented with sudden onset breathlessness. CXR is done and shown below?



- a) Foreign body aspiration
- b) Pneumothorax
- c) Status Asthmaticus
- d) Hydropneumothorax



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Q141. An alcoholic male presents with fever, gradual weight loss. CXR done is shown below?



- a) Miliary T.B.
- b) Silicosis
- c) Cirrhosis with Hepato-pulmonary syndrome
- d) Hemosiderosis

Q142. FDG PET Scan. All statements are true except ?

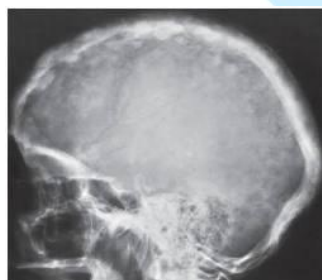
- a) FDG PET utilises PET Gantry in place of Gamma Camera
- b) The uptake of FDG is variable in Heart
- c) FDG PET uptake is a marker for malignancy and therefore used in staging of tumours
- d) FDG PET shows no uptake in carcinoid tumours & broncho-alveolar cancer

Q143. Identify the pathology shown below?



- a) Inverted Papilloma
- b) Juvenile Naso-pharyngeal Angiofibroma
- c) Rhinosporidiosis
- d) Mucormycosis

Q144. An x-ray skull of a middle-aged man with generalised back pain, gradual hearing loss is shown below. What is the likely diagnosis?



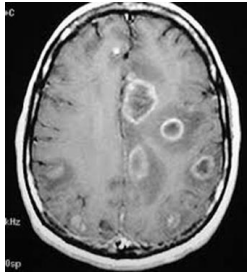
- a) Metastasis from thyroid cancer
- b) Multiple Myeloma
- c) Pagets Disease
- d) Thalassemia



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Q145. A HIV patient with reduced CD-4 counts to 80 cells/ μL presents with fever, altered sensorium and seizures? Contrast enhanced MRI Brain is done and shown below. What is the diagnosis?



- a) HIV Encephalopathy
- b) Toxoplasmosis
- c) Neurocysticercosis
- d) Metastasis

Q146. A 45-year-old woman is undergoing elective abdominal surgery under general anesthesia. The anesthesiologist titrates anesthetic depth using a Bispectral Index (BIS) monitor. Which BIS value range best ensures an adequate level of general anesthesia for surgery?

- a) 80-100
- b) 60-80
- c) 40-60
- d) 20-30

Q147. A 58-year-old man suddenly collapses in the hospital corridor. A nurse immediately activates a Code Blue and reaches him within seconds. The patient is unresponsive. On assessment, he has abnormal respirations with a palpable carotid pulse. What is the most appropriate immediate action by the first responder?

- a) Start chest compressions
- b) Provide rescue breaths at 1 breath every 6 seconds
- c) Place the patient in lateral recovery position and monitor
- d) Reassess breathing and pulse simultaneously for 10 seconds

Q148. Given below is an airway device used during general anesthesia. Consider the following statements:



- i. Provides a higher oropharyngeal seal pressure
- ii. Contains a gastric drainage channel
- iii. Reduces the risk of gastric insufflation and aspiration



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- iv. Acts as a definitive airway with complete protection against aspiration
- v. Requires direct laryngoscopy for insertion

Which of the following statements is/are correct?

- a) i, ii, iii
- b) ii, iv, v
- c) i, ii, iii, iv
- d) iii, iv, v

Q149. A topical anesthetic preparation used before pediatric venipuncture consists of two local anesthetics combined in equal concentration. The combination allows both agents to exist in liquid form at room temperature, enhancing penetration through intact skin. What is the primary reason for combining these two drugs in this preparation?

- a) To prolong duration by protein binding
- b) To produce a eutectic mixture that lowers melting point and enhances dermal absorption
- c) To reduce systemic toxicity by competitive metabolism
- d) To produce faster hepatic metabolism

Q150. During general anesthesia, the anesthesiologist uses a specialized electrically powered vaporizer that is heated and pressurized and injects anesthetic vapor directly into the fresh gas flow instead of using a conventional variable-bypass vaporizer. Which inhalational anesthetic agent is being administered?

- a) Desflurane
- b) Sevoflurane
- c) Halothane
- d) Isoflurane

ANSWERS

1	b
2	c
3	c
4	a
5	d
6	b
7	a
8	b
9	c
10	c
11	a
12	b
13	b
14	b
15	b
16	b
17	b

18	c
19	b
20	c
21	b
22	c
23	c
24	b
25	b
26	b
27	a
28	b
29	b
30	b
31	c
32	d
33	c
34	b

35	a
36	b
37	b
38	b
39	b
40	b
41	b
42	b
43	b
44	b
45	b
46	c
47	c
48	b
49	d
50	d
51	d

52	c
53	c
54	b
55	c
56	d
57	d
58	b
59	c
60	d
61	d
62	d
63	b
64	d
65	d
66	c
67	c
68	d

69	c
70	d
71	c
72	b
73	b
74	d
75	d
76	c
77	b
78	b
79	b
80	c
81	c
82	c
83	b
84	a
85	b



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86	b
87	b
88	c
89	b
90	c
91	a
92	d
93	b
94	b
95	a
96	c
97	a
98	d

99	c
100	a
101	d
102	c
103	b
104	b
105	b
106	a
107	b
108	c
109	d
110	b
111	b

112	b
113	b
114	d
115	a
116	c
117	b
118	a
119	b
120	c
121	d
122	c
123	b
124	c

125	b
126	b
127	c
128	d
129	b
130	a
131	d
132	d
133	c
134	b
135	d
136	d
137	c

138	a
139	b
140	a
141	a
142	c
143	b
144	c
145	b
146	c
147	b
148	a
149	b
150	a